

HOUSE No. 4669

The Commonwealth of Massachusetts

HOUSE OF REPRESENTATIVES, November 5, 2025.

The committee on Mental Health, Substance Use and Recovery, to whom was referred the petition (accompanied by bill, House, No. 2226) of John Francis Moran and Carmine Lawrence Gentile for legislation to establish a special commission (including members of the General Court) to study the availability of the continuum of care services for persons with substance use disorder, reports recommending that the accompanying bill (House, No. 4669) ought to pass.

For the committee,

MINDY DOMB.

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In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act relative to studying and improving the continuum of care for persons with mental health, substance use and co-occurring mental health and substance use disorders.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. (a) Notwithstanding any general or special law to the contrary, the
2 executive office of health and human services shall study the continuum of care for persons with
3 behavioral health conditions, including, but not limited to, mental health, substance use or co-
4 occurring mental health and substance use disorder, and the feasibility of and potential need for
5 creating regional facilities for this purpose.

6 (b) The study shall examine and include recommendations on: (i) the current regional
7 availability of and strategies for overcoming barriers to care and care gaps in behavioral health
8 services, including, but not limited to mental health, substance use and co-occurring mental
9 health and substance use disorder services, acute treatment services, clinical stabilization
10 services, transitional support services, residential recovery home services and outpatient
11 treatment, including, but not limited to, medication assisted treatment; (ii) the impact on
12 communities in which current services and programs are located; (iii) institutional barriers that
13 individuals face in accessing or progressing through the treatment continuum, including

behavioral and physical health needs and acuity, funding, treatment costs and rates, program space and capacity, access to transportation and transit options, harm reduction programs and services, accessibility, housing availability and any other barriers caused by unconnected services or a lack of continuum of care within the current recovery process; and (iv) the creation of an enhanced service architecture that addresses such gaps and barriers, including the need for regional continuum of care facilities for the treatment of mental health and substance use disorders, including co-occurring mental health and substance use disorders, and the optimal capacity at each level of care to ensure individuals can efficiently access and move through each level of care as clinically necessary.

(c) The study shall be conducted with input from relevant stakeholders, including, but not limited to: (i) professional associations and organizations and health care providers with expertise in the provision of care to people with substance use disorder and co-occurring substance use and mental health conditions; (ii) professional associations and organizations and individuals with expertise in the treatment of substance use disorder, co-occurring substance use and mental health conditions and equity in access to health care; (iii) behavioral health professionals with expertise in providing culturally-competent care to people with substance use disorder and co-occurring substance use and mental health conditions; (iv) individuals who have lived experience of seeking or receiving substance use disorder and co-occurring substance use and mental health services or treatment; (v) executive agencies of the commonwealth or its political subdivisions; and (vi) other relevant stakeholders identified by the secretary; provided, however, that the executive office may consult with external experts or other third-party entities when the office lacks the specific scientific, medical or technical expertise necessary for the performance of its responsibilities under this section; and provided further, that the executive

37 office shall disclose when such external expert or third-party entity contributes to its analysis and
38 reporting and the identity of such external expert or third-party entity.

39 (d) Not later than 1 year from the effective date of this act, the executive office shall
40 submit a detailed report with its findings and recommendations, along with drafts of any
41 legislation necessary to carry out such recommendations and any additional information deemed
42 relevant by the executive office, to the clerks of the house of representatives and the senate, the
43 joint committee on mental health, substance use and recovery, the joint committee on health care
44 financing and the governor.

45 (e) Not later than 1 year following submission of the first report and by December 31
46 annually thereafter, the executive office shall submit a report detailing the commonwealth's
47 progress in implementing the recommendations contained in the preceding report, including any
48 barriers to the implementation of such recommendations and any additional recommendations to
49 address such barriers, to the clerks of the house of representative and the senate, the joint
50 committee on mental health, substance use and recovery, the joint committee on health care
51 financing and the governor.